This is a portion of the eBook at doi:10.7551/mitpress/14895.001.0001

10 Women's Economic and Social Vulnerability to the Pandemic: Lessons from Bangladesh, Kenya, and Nigeria

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Introduction

At the beginning of the COVID-19 pandemic, quantitative evidence showed that women around the world were especially vulnerable to the economic and social crises posed by school closures and lockdowns. Women increasingly were left to satisfy the growing demand for unpaid work at home (Del Boca et al. 2020; Agarwal 2021). Many exited the labor force (Seck et al. 2021; Kugler et al. 2023) and struggled with their mental health (Zamarro and Prados 2021; Xue and McMunn 2021). Patterns of economic precarity have also been associated with increased violence against women (Peterman et al. 2020; Agüero 2021). Although the narrative in earlier work focused mainly on the plight of women in high-income countries, an emerging literature in low-income countries painted a stark picture on the state of food insecurity (Josephson et al. 2021). Declines in income-generating activities affected not only the purchasing power of households but likely women's food consumption because of inequitable practices around food allocation (Kumar and Quisumbing 2013; D'Souza and Tandon 2019; Brown et al. 2021) and the weakening of supportive informal networks (Kumar et al. 2018).

In this chapter, we build on previous work in measuring the medium-term changes in women's outcomes to unravel which aspects of their well-being did and did not recover from initial losses. Specifically, we use data from women and men surveyed in Bangladesh, Kenya, and Nigeria at two time periods: an initial survey round conducted from November 2020 to January 21, 2021 (year 1) and another conducted during approximately the same months a year later in 2021–2022 (year 2). A disadvantage of the timing of our longitudinal study is that the first round of the survey took place months after the start of the COVID-19 pandemic. As such, we do not have prepandemic baseline data to really understand how the global implications of the pandemic influenced the well-being of our sample in terms of effects driven by inflation in market prices for staples or manufactured items. Our measure of recovery in essence compares the extent that beneficial outcomes were positively affected (or negative outcomes were reduced) in year 2 relative to year 1. We argue that comparisons of outcomes in years 1 and 2 still capture localized consequences of the pandemic, particularly in Kenya and Nigeria, where the enforcement of COVID-19 policies and the number of COVID-19 cases were rather low at the time of our first-round survey (Mueller et al. 2021; Mueller et al. 2022a). However, we are aware that comparisons of the outcomes over our study period might understate the magnitude of the localized effects in Bangladesh, where the number of infections was relatively high at the time of our first round (Abaluck et al. 2021) and school closures and lockdowns were quite stringent (Mueller et al. 2022a).

We also contribute to improvements in the measurement of equity-deserving groups and well-being metrics included in prior analysis (Egger et al. 2021; Josephson et al. 2021). First, we interviewed five hundred to eight hundred men in Bangladesh, Kenya, and Nigeria using the same survey instrument to differentiate whether the experience in losses was unique to women. Second, we included measures of well-being that have been ignored in the literature. Of particular interest is the integration of time use data to directly identify the extent to which women substituted paid for unpaid work as the burden of the demand for childcare and domestic services grew within the household. Time use data also allow us to capture the extent to which decisions related to time use led to women being deprived of sleep and leisure activities. We additionally measure how these habit-related changes coincided with changes in women's mental health. In addition to these contributions, we provide evidence of how women's employment and their household security, conflict, and food-related coping strategies were affected by the pandemic.

In what follows, we provide a brief overview of the longitudinal survey data collected in the three countries. We then describe our empirical methodology and reflect on changes in women's time use, employment, food insecurity and well-being amid the pandemic. We conclude with recommendations to policymakers based on our descriptive evidence.

Data

We collected three rounds of data during the pandemic in Bangladesh, Kenya, and Nigeria: round 1 (November 2020 to January 2021), round 2 (March and April 2021), and round 3 (November 2021 to January 2022). We only use data from rounds 1 and 3 in this analysis to provide inferences on the welfare consequences of COVID-19 over the course of a year and render comparisons within the same season or business cycle. The survey and human subject participation were reviewed locally by the National Health Research Ethics Committee of Nigeria in Nigeria (NHREC/01/01/2007), the Institutional Review Board of the BRAC James P. Grant School of Public Health at BRAC University in Bangladesh (IRB-13 October 20–043), and the Maseno University Ethics Review Committee in Kenya (MSU/DRPI/MUERC/00906/20). The study was also authorized by an institutional review board committee at Simon Fraser University in agreement with Arizona State University (FWA 00009102; IRB registration number IRB00000128) and the University of Hong Kong. The interviews were performed over mobile phones, and the sampling frame was achieved through the extension of a previous survey in Bangladesh and random digit dial methods in Kenya and Nigeria. The random digit dial approach consisted of purchasing numbers from third-party distributors and then randomly reaching a predetermined quota of men and women to create the national sample. We targeted roughly 1,200 women and 800 men at baseline. Respondents were surveyed if they met the following eligibility criteria: (1) they were at least eighteen years old; (2) they expressed a willingness to be resurveyed in the future and provided follow-up contact information (a phone number, their first name, and date of birth to verify their identity); and (3) they spoke one of the languages familiar to the enumerators on the team (e.g., English or Swahili in Kenya). Mueller et al. (2022a) provide greater detail on the survey design, the sample characteristics, and the national representativeness of data collected via random digit dial.

Survey Samples

The initial samples in round 1, in year 1, consisted of 1,822 individuals (914 men and 908 women) in Bangladesh, 2,038 individuals (744 men and 1,294 women) in Kenya, and 1,969 individuals (825 men and 1,144 women) in Nigeria. By round 3 (year 2), attrition was around 17 percent in Bangladesh, 33 percent in Kenya, and 35 percent in Nigeria. Our analysis relies on the responses of women and men who completed all three survey rounds: 1,547 individuals (793 men and 754 women) in Bangladesh, 1,367 individuals (524 men and 843 women) in Kenya, and 1,280 individuals (570 men and 710 women) in Nigeria. When comparing the attributes of the men and women who are included in our sample versus those who did not complete all three survey rounds, we find very few differences in Bangladesh. We find greater retention of educated (Nigeria) and older individuals (Bangladesh and Nigeria). Married women also tended to remain in the sample more frequently over time in Kenya and Nigeria. Our analysis used sampling weights in the construction of the summary statistics to resemble the national population more closely. In Kenya and Nigeria, the sampling weights provide an additional adjustment to correct for attrition. Mueller et al. (2022a) provide more information regarding the construction of sampling weights.

We provide a brief overview of the characteristics of our sample. The average ages of individuals in our samples at round 1 were forty-one and thirty-eight years old for men and women, respectively, in Bangladesh, twenty-eight years old for men and twenty-nine years old for women in Kenya, and thirty-eight and thirty-six years old for the men and women, respectively, in Nigeria. The percentages of men and women who had completed secondary education or higher at round 1 in Bangladesh are 32 percent and 28 percent, respectively; the percentages for those who had attended secondary education or higher at round 1 in Kenya are 88 percent and 85 percent, and in Nigeria, 91 percent and 88 percent. Thirty-eight percent of men and 36 percent of women were married in Kenya along with around 60 percent of both men and women in Nigeria, while the numbers are an astounding 93 percent of men and 72 percent of women in Bangladesh. The number of children (zero to eighteen years old) reported living in the respondents' households was around two for men and women in Bangladesh and Kenya, and three for men and women in Nigeria.

Outcomes Used in the Analysis

We summarize the main outcomes of interest in table 10.1 classified under themes of time use and employment, food insecurity, and well-being. We implemented a time use module where we asked individuals to recall how much time they had spent on the following activities in the last twenty-four hours: (1) school (including homework) and paid and unpaid work for wage labor or business; (2) getting health services for themselves or other household members; (3) cooking or other domestic work (including cleaning

Table 10.1
Summary of outcomes

Outcome variable	Description
Time use	
Work	Time used in the past 24 hours on school (including homework), work for salary or wages, or paid or unpaid work for business (farming, livestock, fishing, or nonagricultural activity)
Health	Time used in the past 24 hours on obtaining health services for self or other household members
Domestic work	Time used in the past 24 hours on cooking or other domestic work (including cleaning and fetching wood or water)
Child or adult care	Time used in the past 24 hours on unpaid care for children (including helping children with studies or schoolwork) or on unpaid care for adults (sick or elderly)
Shopping	Time used in the past 24 hours on shopping or buying goods
Travel	Time used in the past 24 hours on traveling and commuting
Social	Time used in past the 24 hours on religious activities or prayers and religious services, social activities, social media (Facebook, Instagram, Twitter), hobbies, or leisure (e.g., watching TV, listening to the radio, reading, exercise)
Sleeping	Time used in the past 24 hours on sleeping and resting or on personal care (bathing, grooming)
Eating	Time used in the past 24 hours on eating and drinking
Job and employment	
Employment	Individual worked for someone else or for themselves for pay for one or more hours in the past seven days
Food security	
Food-insecure	Household experienced not enough food or money to buy food in the past seven days
Mental health	
Probable anxiety	Has generalized anxiety disorder combined score greater than or equal to 3
Probable depression	Has patient health questionnaire combined score greater than or equal to 3
Security and conflict	
Feel secure	Household felt very or quite secure these days
No argument	Household had no argument in past six months
Argued once or twice	Household argued once or twice in past six months
Argued monthly or worse	Household argued monthly or more often in past six months
More conflict	Household had more conflict after COVID-19

and fetching wood or water); (4) unpaid care for children (including helping children with studies) or adults (sick or elderly); (5) shopping or buying goods; (6) traveling and commuting; (7) social activities (religious and social activities, social media, hobbies and leisure); (8) sleeping, resting, and personal care; and (9) eating and drinking. Minute increments were reported as fractions of hours. The responses from the time use module were used to create nine time use outcomes for the analysis. Employment is captured by a binary variable that assigns a value of 1 to those individuals who reported working at a paid job for at least one hour in the last seven days and 0 otherwise.

We also created outcomes to assess changes in food security, mental health, and security and conflict. We asked the respondents to first reflect on whether there were days (in the last week) when their household did not have enough food or money to buy food. We developed a binary outcome that assigned respondents a value of 1 for living in a food-insecure household if they answered yes to the question. We used questions from the mental health module of the survey questionnaire to develop two additional well-being indicators at the individual level. In the mental health module, we asked whether the individual over the last two weeks had been bothered by any of the following problems: (1) little interest or pleasure in doing things; (2) feeling down, depressed, or hopeless; (3) feeling nervous, anxious, or on edge; and (4) not being able to stop or control worrying. From the responses to these questions, we created two indicators: one of probable anxiety using the Generalized Anxiety Disorder (GAD)-2 screening tool and the other one demonstrating probable depression using the validated Patient Health Questionnaire (PHQ)-2 (Löwe et al. 2005; Kroenke et al. 2007; Plummer et al. 2016). Both assessments have possible scores ranging from 0 to 6. We created binary outcomes based on whether the values were greater than 3, which is indicative of probable anxiety and depression.

Finally, four outcomes were created based on the questions in the security and conflict module of the questionnaire. We created an outcome regarding whether the individual felt secure based on whether they indicated feeling very or quite secure these days compared to not very or not at all secure. We also borrowed a question designed by Peterman (2020) to examine the extent to which intrahousehold conflict had changed in Kenya and Nigeria:

> When people live together in the same household, they usually share both good and bad moments. And it is normal for people who live together to have arguments. How often in the last six months would you say that people in your household have argued or have had some sort of conflict among themselves?

Individuals could respond (1) never, (2) once or twice, (3) monthly, (4) weekly, (5) daily, (6) don't know, or (7) NA (for respondents who lived alone in the past six months). From the answers to this question, we created three binary indicators to reflect the proportion of individuals living in households without arguments, with arguments once or twice, and with more than one or two arguments. A final well-being indicator was constructed based on a follow-up question that asked the surveyed individuals to reflect on whether they believed conflicts within the household had increased since COVID-19. We created a binary outcome from those responses where a person received a value of 1 if they indicated that arguments were more frequent than before COVID-19 and a value of 0 if not.

Empirical Methodology and Results

In this section we describe how the outcomes changed for women in these three countries over the course of twelve months during the pandemic and then determine whether those changes were different from those experienced by men. Our approach was to first compare the mean values of the outcomes reported during the first and second years. We then discuss which differences are statistically meaningful, drawing from the results of t statistics reported in tables 10.2 and 10.3 that test whether those deviations are significant. Finally, we discuss whether these changes were uniquely experienced by women, also based on t tests that determine whether the change in the mean outcomes over time differ between the men and women respondents (tables 10.2 and 10.3). In what follows, we first present the results for time use and employment and then those for indicators of well-being (food security, mental health, and intrahousehold conflict).

Time Use and Employment

Figure 10.1 illustrates how the amount of time women spent on different activities changed over a one-year period during the pandemic. In table 10.2, we focus on the activities on which women spent most of their time: education and paid work, domestic work, social activities, sleeping, and childcare. The inclusion of childcare activities was motivated by the anticipated effects of disruptions in schooling schedules and barriers to childcare services during periodic curfews and lockdowns. To complement the time use outcomes, in table 10.2 we also summarize the changes in the proportion of women who obtained paid employment.

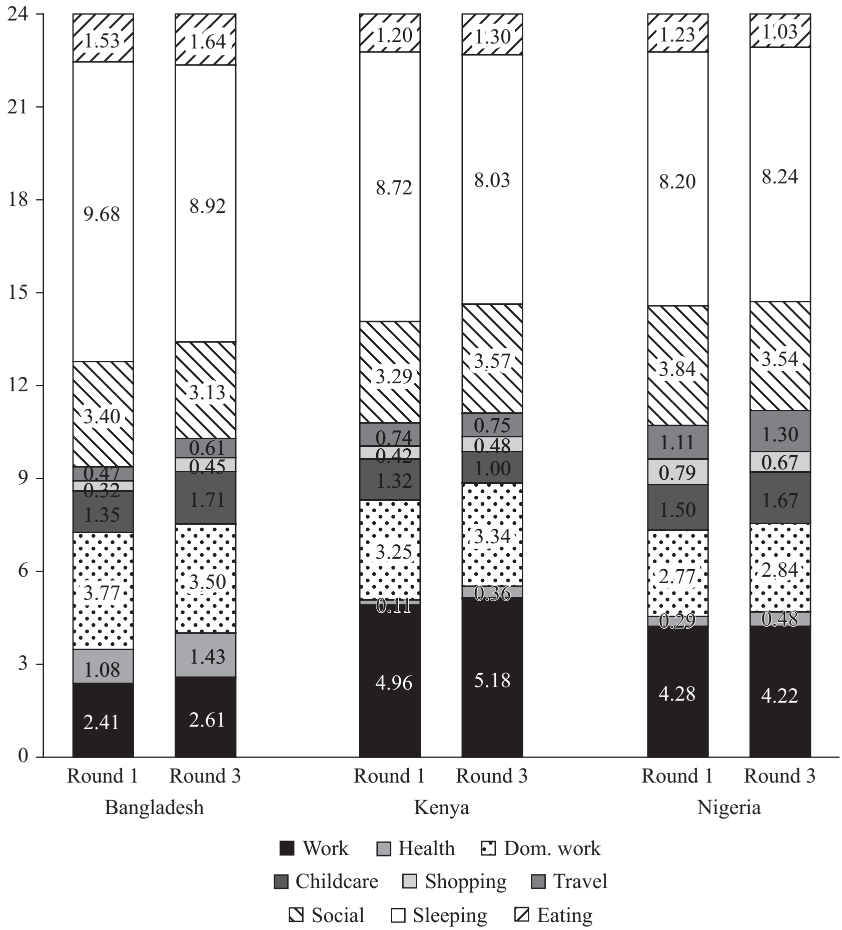

Figure 10.1

Changes in time use during the pandemic for women in Kenya, Nigeria, and Bangladesh.

The results indicate a slight increase in time spent in childcare over the year in Bangladesh (0.35 hours). Women in Kenya appeared to recover 0.32 hours devoted to childcare compared to round 1, which might have been affected by policies at the beginning of the pandemic (Mueller et al. 2022b). This decline seems to correspond with Kenyan women spending more time on paid work. For example, in year 2, women in Kenya report spending 0.32 hours less on childcare in a day and 0.22 hours more on paid work. However, the latter figure is not statistically meaningful. While the number of hours spent on domestic activities remained the same over the year for women in

Kenya and Nigeria, they declined by 0.27 hours for women in Bangladesh. In all three countries, there are no statistically meaningful changes in the number of hours women worked or in the proportion of women employed. Any gains in domestic work or childcare in Bangladesh and Kenya might have come at the expense of sleeping. Women in Bangladesh and Kenya spent 0.76 and 0.69 fewer hours sleeping, respectively, in comparison to round 1. Both of these trends could possibly correspond with a degradation in other aspects of well-being discussed in the next subsection.

Perhaps of broader interest is the extent to which these pandemic consequences were experienced by women alone. In columns 5, 10, and 15 of table 10.2, we compare the change in outcomes by gender and determine whether they are statistically meaningful according to the t test. Men and women faced the same experiences with respect to employment, with one exception: Bangladeshi men appeared to obtain fewer employment hours than women (0.47 hours).

Norms around childrearing and COVID-related policies might have created a unique burden for women in domestic and childcare activities. The declines in unpaid childcare as the pandemic progressed were entirely concentrated among women in Kenya. In the meantime, there was little change in the efforts devoted to domestic work and childcare in Nigeria. The analysis in Bangladesh illustrates that both men and women benefited over time in that they spent less time on domestic work, but they faced an increase in time spent on childcare. The magnitudes of these losses and gains in the various activities were no greater for women relative to men.

Food Insecurity and Well-Being

By the end of the study, we witnessed that food security improved for women relative to the first year (table 10.3). The proportion of women living in food-insecure households declined by 9 percent in Bangladesh, 13 percent in Kenya, and 8 percent in Nigeria. Our indicators at the household level show that men and women were progressing toward similar food security statuses. Changes in the tendency to be food-insecure do not statistically differ by gender according to the t tests reported in columns 5, 10, and 15 of table 10.3.

We next turn to the changes in women's mental health. Figure 10.2 breaks down the average anxiety (GAD-2) and depression (PHQ-2) scores calculated for women based on their survey responses at the start and end of the study period. All mental health scores worsened in the year 2 for women in Kenya

Table 10.2
Changes in time use and employment outcomes by gender in Kenya, Nigeria, and Bangladesh

	Bangladesh					Kenya					Nigeria				
	Women			Men		Women			Men		Women			Men	
	(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)	(13)	(14)	(15)
	R3	R1	R3-R1	R3-R1	(4)—(3)	R3	R1	R3-R1	R3-R1	(9)—(8)	R3	R1	R3-R1	R3-R1	(14)—(13)
	Mean/	Mean/	Mean/	Mean	Mean	Mean/	Mean/	Mean/	Mean	Mean	Mean/	Mean/	Mean/	Mean	Mean
	SE	SE	t test		t test	SE	SE	t test		t test	SE	SE	t test		t test
Time use															
Education and work	2.61 [0.13]	2.41 [0.13]	0.20	–0.43**	–0.63***	5.18 [0.28]	4.96 [0.28]	0.22	1.01**	0.79	4.22 [0.19]	4.28 [0.20]	–0.05	0.48*	0.53
Domestic work	3.50 [0.08]	3.77 [0.09]	–0.27**	–0.39***	–0.12	3.34 [0.13]	3.25 [0.13]	0.09	–0.11	–0.20	2.84 [0.10]	2.77 [0.10]	0.07	0.00	–0.07
Unpaid childcare	1.71 [0.06]	1.35 [0.08]	0.35***	0.36***	0.01	1.00 [0.08]	1.32 [0.11]	–0.32**	0.07	0.39*	1.67 [0.10]	1.50 [0.10]	0.17	–0.01	–0.18
Social activities	3.13 [0.08]	3.40 [0.09]	–0.27**	0.11	0.37***	3.57 [0.14]	3.29 [0.15]	0.28	–0.49*	–0.77**	3.54 [0.12]	3.84 [0.14]	–0.30	–0.45**	–0.16
Sleeping	8.92 [0.08]	9.68 [0.08]	–0.76***	–0.44***	0.31**	8.03 [0.15]	8.72 [0.15]	–0.69***	–1.13***	–0.44	8.24 [0.15]	8.20 [0.17]	0.04	0.23	0.19
Employment															
Paid work	0.15 [0.02]	0.13 [0.02]	0.02	0.01	–0.01	0.58 [0.03]	0.50 [0.03]	0.08	0.06	–0.02	0.55 [0.03]	0.57 [0.03]	–0.01	–0.01	–0.00
N	754	754		793		843	843		524		710	710		570	

Note: Standard errors in brackets. *** $p<0.01$, ** $p<0.05$, * $p<0.1$. The t tests in columns 3, 8, and 13 use the sampling weights in the respective rounds (e.g. baseline in R1 and R3 weights accounting for attrition in R3). Similarly, the corresponding t tests in columns 4, 9, and 14 for the male sample use the sampling weights in the respective rounds. The t statistics presented in columns 5, 10, and 15 are based on calculations that compare the change in outcomes by gender, where the sampling weights applied are only for R3.

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Table 10.3
Changes in food insecurity and well-being outcomes by gender in Kenya, Nigeria, and Bangladesh

	Bangladesh					Kenya					Nigeria				
	Women			Men		Women			Men		Women			Men	
	(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)	(9)	(10)	(11)	(12)	(13)	(14)	(15)
	R3	R1	R3-R1	R3-R1	(4)—(3)	R3	R1	R3-R1	R3-R1	(9)—(8)	R3	R1	R3-R1	R3-R1	(14)—(13)
	Mean/ SE	Mean/ SE	Mean/ t test	Mean	Mean t test	Mean/ SE	Mean/ SE	Mean/ t test	Mean	Mean t test	Mean/ SE	Mean/ SE	Mean/ t test	Mean	Mean t test
Food security															
Household food-insecure	0.19 [0.02]	0.28 [0.02]	−0.09***	−0.13***	−0.03	0.46 [0.03]	0.60 [0.03]	−0.13***	−0.11*	0.02	0.42 [0.03]	0.51 [0.03]	−0.08**	−0.09**	−0.01
N	754	754		793		843	843		524		710	710		570	
Mental health															
Probable anxiety	0.03 [0.01]	0.13 [0.02]	−0.10***	−0.07***	0.03	0.20 [0.03]	0.06 [0.01]	0.14***	0.11***	−0.03	0.21 [0.02]	0.11 [0.02]	0.10***	0.09***	−0.01
Probable depression	0.36 [0.02]	0.43 [0.02]	−0.07**	−0.03	0.05	0.24 [0.03]	0.06 [0.01]	0.18***	0.11***	−0.07*	0.27 [0.02]	0.15 [0.02]	0.13***	0.10***	−0.03
N	754	754		793		843	843		524		710	710		570	
Security and conflict															
Feel secure	0.88 [0.01]	0.57 [0.02]	0.32***	0.39***	0.08**	0.83 [0.03]	0.73 [0.03]	0.10***	0.14***	0.03	0.70 [0.02]	0.65 [0.03]	0.05	0.07**	0.02
N	754	754		792		843	843		524		710	710		570	
Household had no arguments						0.39 [0.04]	0.31 [0.03]	0.08	0.15**	0.07	0.41 [0.03]	0.44 [0.03]	−0.03	−0.01	0.02
Household argued once or twice						0.28 [0.03]	0.32 [0.03]	−0.04	−0.02	0.01	0.30 [0.03]	0.33 [0.03]	−0.03	−0.10**	−0.07
Household argued monthly or more						0.33 [0.03]	0.37 [0.03]	−0.04	−0.12*	−0.08	0.29 [0.02]	0.23 [0.02]	0.06*	0.11***	0.04
N						738	738		393		614	614		447	
More conflict since COVID-19	0.13 [0.02]	0.33 [0.03]	−0.20***	−0.14***	0.06	0.66 [0.03]	0.53 [0.03]	0.13***	−0.00	−0.14*	0.33 [0.03]	0.47 [0.03]	−0.15***	−0.14***	0.01
N	590	590		655		782	782		444		589	589		455	

Note: Standard errors in brackets. *** $p<0.01$, ** $p<0.05$, * $p<0.1$. The t tests in columns 3, 8, and 13 use the sampling weights in the respective rounds (e.g., baseline in R1 and R3 weights accounting for attrition in R3). Similarly, the corresponding t tests in columns 4, 9, and 14 for the men's sample use the sampling weights in the respective rounds. The t statistics presented in columns 5, 10, and 15 are based on calculations that compare the change in outcomes by gender, where the sampling weights applied are only for R3.

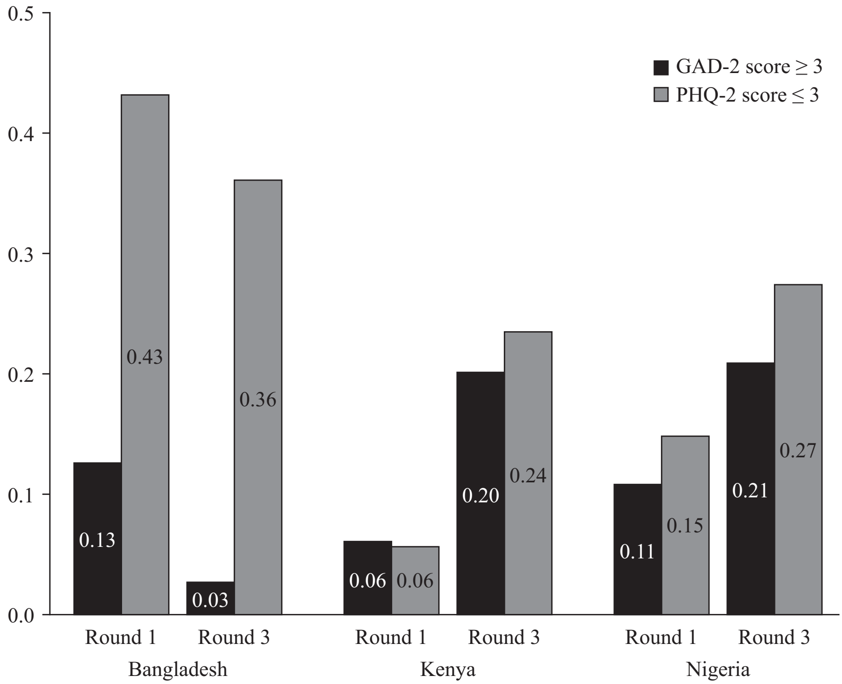

Figure 10.2
Changes in mental health status during the pandemic for women in Kenya, Nigeria, and Bangladesh.

and Nigeria. The proportion of women with probable anxiety and depression declined in year 2 for women in Bangladesh. The t statistics in columns 3, 8, and 13 in table 10.3 illustrate that the differences portrayed in figure 10.2 are statistically significant according to the t tests. The mental health figures illustrate that there are no gendered differences in the changes in anxiety. The increase in the proportion of women who became depressed by year 2 in Kenya was 18 percent, which is 7 percent more than the growth in depression among men. The short-term consequences on food insecurity (Mueller et al. 2022a) and time use and employment (Mueller et al. 2022b) may have created a mental toll on women in the long term in Kenya. Similarly, disruptions in sleeping patterns among Kenyan women may have exacerbated their psychological stress, according to the recent body of COVID-19 literature (Alimoradi et al. 2021; Alimoradi 2022).

A final set of well-being indicators considered in the analysis comprised metrics related to security and conflict. With the exception of women in Nigeria, women reported feeling more secure during the second survey round relative to the first year of the pandemic. Feelings were relatively similar across genders in Kenya. In Kenya, women reported having fewer arguments at home as time went on, but a greater number reported having more conflicts than prior to COVID-19. These sentiments were not consistent with those of men. In Bangladesh, a higher proportion of men, relative to women, felt more secure at the end of the study. Perhaps conflicts within the household might additionally contribute to the disruptions in sleeping patterns and the overall toll on women's mental health.

The reports on security and conflict from respondents in Nigeria and Bangladesh offer a few puzzling results. For example, both men and women in Nigeria consistently reported arguing more at the end of the study period relative to round 1, but less than prior to COVID-19. In Bangladesh, both men and women report having fewer arguments than before COVID-19. One potential explanation for the inconsistent information is that measurement error may increase on self-reported information as the recall period increases over time. Here, the recall period refers to since COVID-19, a period that grew by year 2. Another possible rationale may be that there are other temporal dimensions that contribute to tension in the household that we failed to capture in this study. This is one drawback in having to conduct a phone interview using a short survey instrument and not having interviewed the respondents prior to COVID-19. Not only do we lack information about the well-being of the respondents prior to COVID-19 to have an objective measure prior to the pandemic, but we had to trade off asking detailed questions pertaining to well-being with other crucial questions related to the political, economic, and social factors that have the potential to exacerbate tensions with the household prior to the study.

Conclusion

Gendered norms around the allocation of unpaid and paid work stymie women's economic empowerment and well-being globally. The health crisis appears to have exacerbated these gender inequities. Women were expected to bear the increased burden of domestic work and childcare driven by school closures and mobility restrictions, while their ability to outsource these activities through formal and informal channels became limited. These features of the pandemic reinforced an already low rate of labor force participation among women. As norms constrain women's economic potential in the long term, prioritizing campaigns that shift perspectives around caregiving and domestic work may reduce gender inequities and promote economic growth in all contexts (Hsieh et al. 2019).

The descriptive evidence suggests that the type of programs required to address inequities in the short term will vary by context and depend on an alliance of social protection policies, educational reforms, and health interventions (Hidrobo et al. 2020). While the growing evidence in relation to the suspension of COVID-19 policies has demonstrated that relaxation has allowed children to access their meals at school (Abay et al. 2021) and allowed other household members to spend more time at work (Mueller et al. 2022b), observed declines in mental health, disruptions in sleeping patterns, and the rise in arguments within the household continued. Power relations may operate to reproduce inequality but also challenge it at times. Kenyan women were able to recover the time spent on unpaid work to increase employment. Fewer changes were observed among women in Nigeria. In the meantime, men and women faced similar disruptions in their employment in Bangladesh, yet recovery was worse for men than women. Improvements among women coincided with more harmonious relations in the household. This suggests that existing health institutions should be strengthened in these countries to incorporate affordable psychological services for mothers and couples (Ajayi et al. 2021). Ajayi et al. (2021) note innovative ways of delivering these services through tele-consulting or leveraging the existing network of community health workers.

Childcare constraints may continue to be responsible for inequities in labor force participation that have existed since before COVID-19 and are ongoing today (Bidisha et al. 2022). Governments should expand early childhood educational programs to reduce the burden of childcare on women (Tang et al. 2021). This can reduce the variability in access that results from a reliance on informal childcare networks and efficiently target the most vulnerable groups of women, such as mothers of young children from liquidity-constrained households.

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This is a section of doi:10.7551/mitpress/14895.001.0001

Feminism and COVID-19

How Women Fare in the Face of a Global Crisis

Edited by: Julia Smith, Clare Wenham

Citation:

Feminism and COVID-19: How Women Fare in the Face of a Global Crisis
Edited by: Julia Smith, Clare Wenham
DOI: 10.7551/mitpress/14895.001.0001
ISBN (electronic): 9780262385695
Publisher: The MIT Press
Published: 2026

The open access edition of this book was made possible by generous funding and support from MIT Press Direct to Open

The MIT Press

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Massachusetts Institute of Technology
77 Massachusetts Avenue, Cambridge, MA 02139
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The MIT Press would like to thank the anonymous peer reviewers who provided comments on drafts of this book. The generous work of academic experts is essential for establishing the authority and quality of our publications. We acknowledge with gratitude the contributions of these otherwise uncredited readers.

This book was set in Stone Serif and Stone Sans by Westchester Publishing Services.

Library of Congress Cataloging-in-Publication Data

Names: Smith, Julia, 1982– editor | Wenham, Clare editor
Title: Feminism and COVID-19 : how women fare in the face of a global crisis / edited by Julia Smith and Clare Wenham.
Description: Cambridge, Massachusetts : The MIT Press, [2026] | Includes bibliographical references and index.
Identifiers: LCCN 2025011050 (print) | LCCN 2025011051 (ebook) | ISBN 9780262554039 paperback | ISBN 9780262385695 pdf | ISBN 9780262385701 epub
Subjects: LCSH: COVID-19 Pandemic, 2020–2023—Social aspects | COVID-19 (Disease)—Social aspects | Women—Social conditions—21st century | Emergency management—Social aspects | Equality
Classification: LCC RA644.C67 G428 2026 (print) | LCC RA644.C67 (ebook)
LC record available at https://lccn.loc.gov/2025011050
LC ebook record available at https://lccn.loc.gov/2025011051

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